# Competitive Landscape: non-small cell lung cancer (EU, 2023)

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## Market Overview

In the EU non-small cell lung cancer market, 4 brands compete for second-line+ patients as of Q3 2024. The segment is shaped by guideline updates, reimbursement decisions and real-world evidence.

The number of branded options in non-small cell lung cancer has grown since 2020, increasing pressure on standard of care positioning.

## Positioning: Keytrava

In the EU non-small cell lung cancer market, Keytrava maintains a 9% market share as of Q3 2024, with the strongest uptake among oncologists.

Sentiment index for Keytrava in EU: 0.33 (scale 0 to 1, 2024). Clinicians view the brand negatively; long-term safety data are still limited.

## Positioning: Opdivra

In the EU non-small cell lung cancer market, Opdivra maintains a 38% market share as of Q4 2024, with the strongest uptake among oncologists.

Sentiment index for Opdivra in EU: 0.29 (scale 0 to 1, 2024). Clinicians view the brand negatively; payer prior-authorisation requirements slow initiation.

## Positioning: Tagrisa

In the EU non-small cell lung cancer market, Tagrisa maintains a 19% market share as of Q2 2024, with the strongest uptake among oncologists.

Sentiment index for Tagrisa in EU: 0.73 (scale 0 to 1, 2024). Clinicians view the brand strongly positively; infusion logistics add to treatment burden.

## Positioning: Lumakro

In the EU non-small cell lung cancer market, Lumakro maintains a 37% market share as of Q1 2024, with the strongest uptake among oncologists.

Sentiment index for Lumakro in EU: 0.50 (scale 0 to 1, 2024). Clinicians view the brand moderately positively; cost and formulary access remain barriers.

## Competitive Dynamics

Lumakro has been gaining ground since its 2020 approval for non-small cell lung cancer, pressuring Keytrava in first-line use.

Price competition, biosimilar entry and head-to-head evidence are expected to reshape formulary position over the next 24 months.

Biosimilar and generic competition in adjacent classes is shaping price expectations, even where Keytrava has no direct substitute.

Respondents expect real-world persistence on Keytrava to be lower than in trials, mainly because of injection-site discomfort and the complexity of managing immune-related adverse events.

Safety monitoring focused on pneumonitis and immune-related adverse events, with dose interruption or discontinuation used for severe adverse events.

Subgroup analyses suggest that benefit with Keytrava is most pronounced in patients with a favourable biomarker profile and a lower comorbidity burden.

The unmet need in non-small cell lung cancer persists in patients who relapse or become refractory, where options are limited and outcomes remain poor.

Sequencing after progression remains an open question; oncologists in United Kingdom reported using a different mechanism of action after Keytrava fails.

Medical education priorities include recognising pneumonitis early, communicating realistic expectations, and coordinating with primary care.

Payer decisions increasingly rely on health technology assessment, requiring evidence of clinical benefit versus the standard of care and cost-effectiveness analyses.

## Outlook

We expect continued consolidation in non-small cell lung cancer. Differentiation will depend on tolerability, dosing convenience and evidence of overall survival or clinical benefit.

The competitive set is evolving as new mechanisms enter phase 3 development, with readouts expected over the next two to three years.

Safety monitoring focused on pneumonitis and rash, with dose interruption or discontinuation used for severe adverse events.

Real-world evidence from registries suggests that outcomes in routine practice are broadly consistent with phase 3 trial results, although persistence and discontinuation rates are higher outside clinical trials.

Access to specialist centres influences adoption: academic centres adopt new therapies faster than community practices, where formulary restrictions and reimbursement criteria apply.

Dose modification guidance for Keytrava is generally followed, but oncologists said that practical monitoring of pneumonitis depends on local capacity and patient travel distance.

Peer-to-peer education and congress data were cited as the most trusted sources when oncologists evaluate new evidence on PD-1 inhibitor agents.

Subgroup analyses suggest that benefit with Keytrava is most pronounced in patients with a favourable biomarker profile and a lower comorbidity burden.

Clinical guidelines in France recommend second-line use of agents in this class, and recent updates have broadened eligibility based on biomarker status and comorbidity profile.

*Synthetic document generated for the Watson prototype. All brand names and figures are fictional.*